

SUMMIT VALE MULTISPECIALTY HOSPITAL

General Hospital Frequently Asked Questions

Synthetic answers about emergency access, appointments, admission, visitors, privacy, services, discharge, billing, and patient support

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Audience	Patients, families, visitors, caregivers, referring clinicians, contact-center staff, and hospital employees	Classification	Synthetic public-facing hospital information reference

Dataset status

This document is a realistic synthetic artifact created to test document ingestion, chunking, retrieval, metadata filtering, reranking, citation, and question-answering workflows. It has not been reviewed or approved for patient care.

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Controlled use

Printed copies are uncontrolled. Users should verify the effective version, approved local order sets, current formulary information, and governing law before applying any content.

1. How to Use This FAQ and Get Urgent Help

This document answers common operational questions but does not diagnose symptoms, replace professional medical advice, or provide a real hospital contact service. [1-3]

Key considerations are: Q: What should I do in a medical emergency? A: Call the local emergency number or go to the nearest emergency department for severe breathing difficulty, chest pressure, stroke symptoms, uncontrolled bleeding, unresponsiveness, seizure, major injury, or another immediately dangerous condition. Q: Can I send an urgent symptom through the patient portal? A: No; portal messages and routine contact forms may not be reviewed immediately and should not be used for time-sensitive or life-threatening concerns. Q: How do I decide between emergency, urgent, and routine care? A: Use emergency services for immediate threats, same-day clinical advice for rapidly worsening but stable concerns, and scheduled care for nonurgent evaluation or follow-up.

Within the SVMH care pathway, Emergency routing takes priority over registration, authorization, price estimation, or selection of a preferred clinician. Emergency Services and Patient Communications maintain consistent public warning language.

Table 1. Illustrative care-routing examples

Situation	Suggested route	Why
Severe chest pain, stroke signs, blue color, unresponsiveness, major trauma	Emergency services now	Delay may cause serious harm
New fever after chemotherapy, worsening asthma despite rescue medicine, repeated vomiting	Same-day direct clinical advice	May require urgent assessment
Stable chronic symptom, preventive visit, routine refill requested early	Scheduled clinic/contact route	Can be addressed through normal workflow
Question about a bill, record, appointment, or visitor rule	Administrative contact route	Clinical emergency resources are not required

Warning
All telephone numbers, addresses, and contact routes in this synthetic dataset are fictional. Do not use them to seek real medical help.

2. Appointments, Referrals, and Choosing a Service

Appointments are scheduled according to the clinical question, urgency, age, requested location, specialist availability, referral requirements, and coverage rules.

Assessment should address the following: Q: Do I need a referral? A: Some specialties and insurance plans require one, while others allow direct scheduling; the contact center can identify the usual route but cannot guarantee payer approval. Q: Can I request a specific doctor? A: A preference can be recorded, although the earliest appropriate appointment may be with another qualified clinician or at another location. Q: What information makes referral review faster? A: Provide the reason for referral, symptom timeline, relevant examination findings, prior treatment, medicines, allergies, test reports, images when needed, and the referring clinician contact.

For routine implementation, Clinical triage reviews referrals that are urgent, incomplete, procedure-specific, age-restricted, or likely to require a different service. Patient Access monitors incomplete referrals, wait times, redirection, repeated contacts, and appointments closed without a documented outcome.

Table 2. Common appointment information

Request	Helpful details	Possible next step
New specialist visit	Referral question, records, prior tests, urgency	Clinical triage or scheduling
Follow-up visit	Treating service, last visit, requested interval, new issue	Return appointment
Procedure/test	Order, indication, preparation needs, authorization	Procedure scheduling
Second opinion	Diagnosis, complete records, images/pathology, specific question	Subspecialty review

3. Changing, Canceling, or Missing an Appointment

Early notice allows the hospital to offer the time to another patient and helps the care team decide whether a delay is clinically safe.

The practical interpretation is as follows: Q: How do I change or cancel a visit? A: Use the scheduling number shown on the confirmation or the authenticated portal and provide the patient name, date of birth, appointment date, service, and preferred alternative. Q: What happens if I arrive late? A: The service may shorten, reschedule, or redirect the visit depending on safety, clinic flow, transportation, interpreter, and procedure preparation. Q: What if I missed an important follow-up? A: Contact the treating service promptly, especially after hospitalization, abnormal testing, medicine change, surgery, or a high-risk diagnosis.

During assessment and treatment, Scheduling notes the reason for cancellation when relevant and sends high-risk delays to the clinical team rather than automatically moving the appointment far into the future. Access leadership reviews no-show patterns for transport, communication, disability, language, cost, and scheduling barriers before applying restrictions.

Practice note
A canceled appointment does not automatically cancel linked laboratory tests, imaging, procedures, transportation, interpreters, or authorizations; verify each related item.

4. Preparing for Arrival, Parking, and Check-in

Preparation reduces delays and helps staff complete identification, consent, medication reconciliation, safety screening, and financial registration accurately.

Relevant clinical features include: Q: What should I bring? A: Bring government or hospital identification when available, insurance information, referral or order, current medicines and allergies, relevant records, mobility or communication aids, and questions. Q: How early should I arrive? A: Follow the confirmation instructions because routine visits, imaging, surgery, laboratory collection, sedation, and first-time registration have different arrival requirements. Q: What should I tell the hospital before arrival? A: Report interpreter, wheelchair, sensory, behavioral, infection, pregnancy, implanted-device, transportation, fasting, or caregiver needs as early as possible.

At each transition of care, Check-in uses at least two patient identifiers and verifies the specific service rather than relying on room location, appearance, or a single printed label. Patient Access and Safety teams audit identification events, duplicate records, registration corrections, accessibility delays, and wayfinding complaints.

- Do not bring large valuables or unnecessary cash.
- Keep medicines in original labeled containers when asked to bring them.
- Use the exact preparation instructions supplied for the scheduled service.

5. Admission, Rooms, and Personal Belongings

Hospital admission occurs when an authorized clinician determines that inpatient or observation-level care is needed and the appropriate administrative status is assigned.

The core elements are: Q: Can I reserve a particular room? A: Preferences may be recorded, but room assignment depends on clinical needs, infection precautions, age, monitoring, staffing, capacity, and available accommodations. Q: What happens to valuables? A: Patients should send valuables home when possible or use the documented hospital valuables process; the hospital cannot treat an unattended bedside item as formally deposited property. Q: Who will coordinate my care? A: The attending clinician and assigned nursing team coordinate the episode with consultants, pharmacists, therapists, Care Management, and other services as needed.

When the guideline is applied in practice, The admission team verifies identity, allergies, medicines, advance-care information, communication needs, emergency contacts, belongings, and the initial plan of care. Hospital Operations reviews delays, inappropriate placements, lost property reports, unresolved medication histories, and failures to identify an accountable clinician.

Table 3. Admission topics to verify

Topic	Patient/family information	Hospital action
Medicines/allergies	Actual products, doses, timing, reactions	Reconcile and clarify discrepancies
Communication	Language, hearing, vision, cognition, preferred contact	Arrange appropriate support
Belongings	Devices, valuables, clothing, assistive items	Document and secure when accepted
Care preferences	Decision-maker, advance directive, cultural or spiritual need	Verify and place in care plan

6. Visitors, Support Persons, and Overnight Stays

The hospital supports family and chosen support persons while balancing patient preference, privacy, infection prevention, safety, rest, and the needs of other patients. [4,5]

Key considerations are: Q: Who may visit? A: The patient may generally identify visitors and a support person, subject to clinically necessary and nondiscriminatory restrictions for safety, infection control, capacity, privacy, or treatment. Q: May a visitor stay overnight? A: Overnight presence depends on the unit, room, patient condition, roommate privacy, age, isolation status, and staff approval under the current visitor policy. Q: Can children visit? A: Children may be permitted when supervised and when the unit can safely accommodate them, but some critical, procedural, behavioral, or isolation settings may restrict visits.

Within the SVMH care pathway, Visitors follow screening, identification, hand-hygiene, personal protective equipment, photography, quiet-hours, and staff-direction requirements. Unit leadership documents significant restrictions and offers reasonable alternatives such as telephone or video contact when feasible.

Warning

A visitor who is threatening, disruptive, intoxicated, refuses required precautions, or interferes with care may be asked to leave and may be subject to security action.

7. Patient Rights, Consent, Communication, and Accessibility

Patients have rights to respectful, nondiscriminatory care, understandable information, participation in decisions, privacy, appropriate communication assistance, and a process for raising concerns. [4-6]

Assessment should address the following: Q: Can I ask questions before agreeing to treatment? A: Yes; informed consent includes the proposed intervention, expected benefits, material risks, alternatives, and the likely consequences of declining when these can reasonably be explained. Q: How do I request an interpreter or disability accommodation? A: Tell scheduling or the care team as early as possible; qualified language assistance and appropriate auxiliary aids are arranged according to need and law. Q: Can a family member make decisions for me? A: A legally authorized representative may act when permitted, while the patient remains involved to the extent possible and documented preferences are respected.

For routine implementation, Staff use teach-back, accessible formats, and qualified interpreters rather than relying on minor children or untrained companions for important medical communication except in a true emergency. Patient Rights, Compliance, and Accessibility leaders review complaints, accommodation delays, consent defects, discrimination concerns, and communication-related safety events.

- Ask for an explanation in everyday language.
- Request a copy of signed consent information when available.
- Tell staff when pain, fear, literacy, hearing, vision, or cognition affects understanding.

8. Privacy, Confidentiality, Medical Records, and the Portal

Health information is used and disclosed for treatment, payment, operations, and other purposes allowed or required by law, with safeguards appropriate to the information and setting. [5,7]

The practical interpretation is as follows: Q: How do I request my medical record? A: Submit the approved request with identity verification and enough detail to identify the patient, dates, facilities, and records needed; fees and timing follow applicable rules. Q: Can the hospital discuss my care with family? A: Staff consider the patient's permission, documented preferences, professional judgment, capacity, and legal authority before sharing information. Q: What should I do if portal information looks wrong? A: Contact the responsible clinical service for urgent medical discrepancies and Health Information Management for formal amendment or record-release questions.

During assessment and treatment, Portal proxy access, adolescent confidentiality, sensitive results, deceased-patient records, subpoenas, research requests, and law-enforcement inquiries follow specialized workflows. The Privacy Officer and Health Information Management monitor unauthorized access, misdirected disclosures, amendment requests, portal proxy risks, and release turnaround.

Table 4. Common health-information requests

Request	Primary route	Important detail
Copy of records	Health Information Management	Specify patient, dates, facility and record type
Correction/amendment	Formal amendment workflow	Explain the disputed entry and requested change
Portal access/proxy	Portal support plus identity verification	Authority and age-specific rules apply
Immediate clinical question	Treating service or urgent care route	Do not rely on routine record-release processing

9. Medicines, Refills, and Hospital Pharmacy Services

Medication safety depends on one accurate list, clear ownership, appropriate monitoring, timely refill requests, and reconciliation whenever care moves between settings.

Relevant clinical features include: Q: Should I bring my home medicines? A: Bring an accurate list or labeled containers when instructed, but do not take personal medicines during admission unless the care team has reviewed and authorized them. Q: How do I request a refill? A: Contact the prescribing clinic or use its authenticated refill route before the supply runs out and provide the medicine, dose, pharmacy, remaining supply, and any recent change. Q: Can the hospital pharmacy fill discharge prescriptions? A: The synthetic outpatient pharmacy may fill eligible prescriptions, check interactions, teach use, and coordinate some assistance, although availability and coverage must be confirmed.

At each transition of care, Controlled medicines, specialty products, compounded medicines, early refills, prior authorization, and products requiring laboratory monitoring may need additional review and cannot be guaranteed immediately. Pharmacy monitors high-alert medicines, reconciliation, discharge access, adverse events, storage, recalls, shortages, and unresolved authorization barriers.

Warning

Do not stop insulin, anticoagulants, seizure medicines, steroids, transplant medicines, or other critical therapy abruptly unless a qualified clinician directs the change or emergency care is required.

10. Laboratory Tests, Imaging, and Diagnostic Results

Diagnostic services require a valid order, correct patient and test identification, appropriate preparation, and a reliable plan for communicating and acting on results. [8,9]

The core elements are: Q: Do I need to fast? A: Only some tests require fasting or medicine adjustments; follow the exact written instructions and ask the ordering service when the direction is unclear. Q: When will results appear? A: Timing varies by test complexity, specimen quality, urgency, outside review, and whether a clinician must interpret the result before release. Q: Who explains an abnormal result? A: The ordering or otherwise designated clinician is responsible for interpretation, next steps, and follow-up, while critical findings are communicated through an urgent closed-loop process.

When the guideline is applied in practice, Before imaging or a procedure, disclose pregnancy possibility, allergies, kidney problems, implanted devices, prior contrast reaction, anticoagulants, diabetes medicines, and inability to lie still or tolerate enclosed spaces. Laboratory, Radiology, and clinical services audit patient identification, specimen quality, radiation and contrast safety, critical-result communication, and unreviewed results.

Practice note

A result marked outside a reference range is not automatically an emergency, and a result inside the range does not rule out illness; interpretation depends on the clinical context.

11. Surgery, Procedures, Sedation, and Day-of-Service Questions

Procedural safety requires confirmation of the patient, procedure, site, consent, preparation, medicines, allergies, test results, transport, and recovery plan.

Key considerations are: Q: Why might a procedure be delayed or canceled? A: Acute illness, unsafe laboratory results, incomplete preparation, unavailable equipment or staff, unresolved consent, no safe transport, or an emergency case may require a change. Q: Which medicines should I stop? A: Use only the individualized preprocedure instructions because anticoagulants, diabetes medicines, supplements, steroids, and other products require different plans. Q: Can I drive home after sedation? A: Usually not; the procedure team will specify the required responsible adult, transport, supervision, activity limits, and warning signs.

Within the SVMH care pathway, Report fever, breathing symptoms, new pregnancy information, missed preparation, medicine changes, wounds, infection, or transportation problems before arrival when possible. Perioperative governance reviews cancellations, site verification, consent, retained-item prevention, sedation events, unexpected transfer, and postprocedure communication.

Warning

Do not guess about fasting or medicine instructions. Contact the procedure team using the confirmed route when directions conflict or have not been received.

12. Infection Prevention, Masks, Isolation, and Hand Hygiene

Precautions are selected according to symptoms, diagnosis, exposure, procedure, patient vulnerability, and current public-health or facility conditions. [8,10]

Assessment should address the following: Q: When should I postpone a routine visit because I am sick? A: Contact the service before arrival for fever, rash, vomiting, diarrhea, new respiratory symptoms, or a known contagious exposure so the visit can be safely routed. Q: Why are staff or visitors wearing protective equipment? A: Gloves, gowns, masks, respirators, eye protection, and room precautions reduce exposure to blood, body fluids, respiratory particles, or specific organisms. Q: What can visitors do to reduce infection? A: Clean hands on entry and exit, follow posted precautions, avoid touching devices or wounds, do not visit when contagious, and use only the patient's bathroom or supplies when directed.

For routine implementation, The care team explains precautions in understandable language and reassesses restrictions when diagnostic information or transmission risk changes. Infection Prevention monitors healthcare-associated infection, exposure response, precaution adherence, hand hygiene, device use, and communication about isolation status.

13. Discharge, Follow-up, Home Care, and Warning Signs

Discharge is a transition of care, not only physical departure, and should leave the patient with an understandable plan, necessary medicines and equipment, follow-up, and a route for questions. [2,11]

The practical interpretation is as follows: Q: What should I know before leaving? A: Confirm diagnoses, medicine changes, wound or device care, diet and activity limits, pending results, warning signs, follow-up appointments, and who to contact. Q: What if I cannot obtain a medicine, equipment, transport, food, or safe help at home? A: Tell the team before discharge so Pharmacy, Care Management, Social Work, therapy, or another service can assess alternatives. Q: Who follows pending test results? A: The discharge documentation should name the responsible clinician or service and the expected communication plan rather than placing the burden only on the patient.

During assessment and treatment, The patient or caregiver uses teach-back to explain the plan and demonstrates any high-risk medicine, device, wound, injection, feeding, or mobility task before departure when appropriate. Transitions leadership reviews readmissions, medication discrepancies, failed referrals, delayed equipment, uncommunicated results, and inability to reach high-risk patients.

Table 5. Discharge checklist

Domain	Confirm before departure	Escalate when
Medicines	Start/stop/change, dose, access, monitoring	Supply or instruction is unclear
Follow-up	Service, date/time frame, referral and transport	High-risk visit cannot be arranged
Home needs	Equipment, wound/device care, food, caregiver, therapy	Home plan is not safe or feasible
Warning signs	Symptoms, action, urgent and emergency routes	Patient cannot explain when to seek help

14. Billing, Insurance, Estimates, and Itemized Statements

A hospital bill may include facility, professional, laboratory, imaging, pharmacy, procedure, supply, and other components that are processed at different times. [12-14]

Relevant clinical features include: Q: Can I receive an estimate? A: Patient Financial Services can provide a good-faith or other available estimate using the scheduled service and known coverage, but the final bill may change when actual care differs. Q: Why did I receive more than one bill? A: The hospital, physicians, anesthesia, radiology, pathology, ambulance, or other entities may bill separately according to contracting and service arrangements. Q: How do I question a charge? A: Request an itemized statement, compare it with the explanation of benefits, identify the specific date or service, and use the documented billing-dispute route.

At each transition of care, Insurance eligibility and authorization are checked when feasible, but verification is not a guarantee of payment and the payer makes the final coverage determination under the plan. Revenue Cycle and Compliance monitor estimate accuracy, billing corrections, duplicate charges, financial-assistance screening, complaints, and legally required notices.

Table 6. Illustrative billing documents

Document	What it shows	What to do with it
Estimate	Expected services and projected patient amount	Review assumptions before scheduled care
Claim	Services submitted to the payer	Track payer processing and requests
Explanation of benefits	Coverage decision, allowed amount, patient share	Compare with provider statement; not usually a bill
Itemized statement	Detailed billed services and adjustments	Use for questions or disputes

15. Payment Methods, Financial Assistance, Collections, and Refunds

Patients may use approved payment methods and may request screening for financial assistance or a payment arrangement when eligible under the hospital’s synthetic policies. [12-14]

The core elements are: Q: What payment methods are accepted? A: The fictional hospital accepts approved card, electronic, check, cash, portal, telephone, and in-person methods, but real credentials must never be sent to any synthetic address or number. Q: How do I apply for financial assistance? A: Contact Patient Financial Services, complete the current application, provide required household and income information, and ask about presumptive or emergency pathways when documents are difficult to obtain. Q: When is a refund issued? A: A verified credit may be refunded after claims, reversals, charge corrections, payment ownership, outstanding balances, and legal or contractual offsets are reviewed.

When the guideline is applied in practice, A billing dispute or assistance application is documented and routed so that collection activity is handled according to the applicable hold, notice, and escalation rules. Finance and Compliance review financial-assistance access, extraordinary collection activity, refund aging, misapplied payments, chargebacks, and complaints.

Practice note

All prices, balances, payment channels, and contact data in this knowledge base are fictional examples for retrieval testing.

16. Children, Adolescents, Guardians, and Caregivers

Care involving minors requires age-appropriate communication, verified authority, attention to confidentiality, weight-based safety, safeguarding, and the child's evolving ability to participate.

Key considerations are: Q: Who may consent for a child? A: A parent, guardian, or other authorized person generally provides permission, subject to emergency, minor-consent, custody, public-health, and other applicable rules. Q: Can an adolescent speak with the clinician privately? A: Private discussion may be offered for developmentally appropriate care while confidentiality and parental access follow law and clinical safety requirements. Q: What should caregivers bring? A: Bring custody or guardianship information when relevant, medicine and allergy details, immunization and growth records, prior reports, devices, school or therapy information, and comfort supports.

Within the SVMH care pathway, Staff verify the adult's identity and authority, involve the child through assent when appropriate, and escalate conflicting custody or safety information before nonemergency decisions. Children's Services and safeguarding leaders review consent questions, confidentiality errors, weight-based medication risk, suspected abuse or neglect, and caregiver access barriers.

Warning

Immediate danger, suspected abuse or neglect, poisoning, severe breathing difficulty, seizure, unresponsiveness, or suicidal crisis requires emergency and safeguarding action.

17. Compliments, Complaints, Safety Concerns, and Ethics Support

Patients and families may raise concerns without fear of retaliation and may request help when communication, safety, rights, treatment choices, or discharge planning remain unresolved. [4-6]

Assessment should address the following: Q: Who should I tell first about a concern? A: When comfortable, tell the bedside nurse, clinician, department manager, or service supervisor so the team can address immediate needs; Patient Relations is available for further review. Q: How do I report a safety concern? A: Describe what happened, where and when it occurred, who was involved, the current risk, and the best contact method; urgent hazards should be reported directly rather than only through a later survey. Q: When can the ethics service help? A: Ethics consultation may support difficult decisions, disagreement about goals or decision-making authority, treatment burden, confidentiality, or values conflicts, while legal advice remains separate.

For routine implementation, Patient Relations acknowledges the concern, records requested communication, coordinates review, protects privacy, and explains available internal or external escalation routes. Hospital leadership trends complaints, grievances, compliments, safety reports, ethics themes, retaliation allegations, and overdue responses for improvement.

18. Synthetic Contact Directory and Final Questions

The following contact routes exist only to create realistic entities for semantic search, metadata filtering, citation, and routing tests; none is connected to a real hospital.

The practical interpretation is as follows: Q: What is the main synthetic contact center? A: The fictional route is +1 (555) 010-2200 for general navigation and +1 (555) 010-2299 for after-hours administrative messaging, neither of which should be used for real care. Q: Which office handles common requests? A: Patient Access handles appointments, Health Information Management handles records, Patient Financial Services handles estimates and bills, and Patient Relations handles complaints and rights questions. Q: What information should I leave in a message? A: Use the minimum necessary information: patient name, date of birth, general reason, service, callback number, and urgency, but never send payment credentials or detailed health information to an unverified address.

During assessment and treatment, Contact-center staff authenticate the caller as required, document the destination and urgency, and use warm transfer or escalation for safety-critical needs. Communications and each department owner test contact routes, update public information, and remove obsolete numbers and instructions.

Table 7. Fictional contact routes for dataset testing only

Synthetic office	Fictional route	Typical topic
Main contact center	+1 (555) 010-2200	Navigation, appointments, general questions
Health Information Management	+1 (555) 010-2230	Records, amendments, portal proxy questions
Patient Financial Services	+1 (555) 010-2240	Estimates, bills, assistance, refunds
Patient Relations	+1 (555) 010-2250	Rights, complaints, compliments, unresolved concerns

Warning
These numbers and all other identifiers in this document are deliberately fictional. In an actual emergency, use the real emergency number for the patient's location.

19. References

References are provided to make the synthetic document realistic and to support citation and provenance tests. They should be checked against the most current source before any real-world use.

[1] Centers for Medicare & Medicaid Services. Conditions of Participation for Hospitals, including patient rights and discharge planning requirements, current electronic edition.

[2] Centers for Medicare & Medicaid Services. Discharge Planning requirements and interpretive guidance for hospitals, current electronic edition.

[3] The Joint Commission. National Patient Safety Goals for the Hospital Program, current edition.

[4] The Joint Commission. Hospital Accreditation Standards addressing rights, responsibilities, communication, and complaint management, current edition.

[5] U.S. Department of Health and Human Services, Office for Civil Rights. HIPAA Privacy Rule guidance for individuals, families, records access, and permitted disclosures.

[6] U.S. Department of Health and Human Services, Office for Civil Rights. Section 1557 nondiscrimination and effective-communication requirements, current guidance.

[7] U.S. Department of Health and Human Services. Individuals' right under HIPAA to access health information, current guidance.

[8] Centers for Disease Control and Prevention. Standard Precautions and infection-control guidance for healthcare settings, current electronic edition.

[9] American College of Radiology. ACR Manual on Contrast Media, current edition.

[10] Centers for Disease Control and Prevention. Core Infection Prevention and Control Practices for Safe Healthcare Delivery in All Settings, current electronic edition.

[11] Centers for Medicare & Medicaid Services. Hospital discharge appeal, beneficiary notice, and care-transition resources, current electronic edition.

[12] Centers for Medicare & Medicaid Services. Hospital Price Transparency requirements and consumer resources, current electronic edition.

[13] Centers for Medicare & Medicaid Services. No Surprises Act and good faith estimate resources for consumers and providers, current electronic edition.

[14] Internal Revenue Service. Requirements for tax-exempt hospital financial assistance and billing and collection policies under Internal Revenue Code section 501(r), current guidance.

Reference status
External guidelines and regulations may change between scheduled reviews. The document owner is responsible for checking high-impact updates and initiating an interim revision when necessary.